

PROGRAM GUIDE

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AAA Program - Overview & Eligibility

Section 01

Eligibility Requirements

- Age 60 or older (55+ for Caregiver Support option)
- Resides in one of the 10 approved Texas counties
- Does NOT have Medicaid
- Meets income eligibility requirements (see AAA referral page for limits)

What AAA Is NOT

- NOT a long-term care solution, it is temporary and time-limited
- NOT extendable, services end and do not continue past the approved period
- NOT a permanent alternative to Private Pay, LTC Insurance, or other payer sources
- Staff must communicate this clearly from the first contact

Approved Service Counties (Texas) Referrals outside these counties are not eligible

Bastrop

Blanco

Burnet

Caldwell

Fayette

Hays

Lee

Llano

Travis

Williamson

Caregiver Support - 55+

Cares for someone 60+ · Misses work to provide care · Has own medical conditions while caregiving

Care Coordination - 60+

Meets eligibility · No Medicaid · Limited access to private pay funds · Age 60+ required

AAA Referral SOP

Step-by-step referral submission process and required form fields

AAA Referral - Submission Process

Section 02

1

STEP 1 • Determine referral type

Identify whether this is Caregiver Support (55+, caring for someone 60+) or Care Coordination (60+, meets income/county/Medicaid eligibility). Confirm eligibility before starting the form

2

STEP 2 • Complete Page 1

Fill in: Date, Referral Source, Name of individual needing services. Ensure ALL applicable service boxes are checked

3

STEP 3 • Complete Page 2 - All asterisk (*) fields required

Full name, DOB, address, gender, phone, ethnicity, race, living situation (lives alone?), income status (at or below poverty level). Confirm top-right checkbox selected. Explain and confirm Release of Information + Client Rights

4

STEP 4 • Complete Page 3

Diagnoses and/or medical conditions, reason services are being requested, emergency or primary contact name and details

5

STEP 5 • Communicate expectations to family

AAA services will end, transition to Private Pay, LTC Insurance, or another payer source must begin early. Never imply extensions are possible

AAA is a temporary program. Set transition expectations with the family from Day 1, do not wait until services are ending.

AREA AGENCY ON AGING (AAA) REFERRAL SOP

A free service funded by the State of Texas to support older adults and their families.



SUPPORT. INDEPENDENCE. DIGNITY.

1 OVERVIEW

The Area Agency on Aging (AAA) is a free service funded by the State of Texas.

- Referrals may only be submitted for individuals residing within our designated service territory.

APPROVED SERVICE COUNTIES

- Bastrop
- Blanco
- Burnet
- Caldwell
- Fayette
- Hays
- Lee
- Llano
- Travis
- Williamson



2 PROGRAM CLARIFICATION (IMPORTANT)

AAA is not a long-term care solution. It is a temporary support program intended to:

- Assist families with identifying long-term care options, or
- Provide short-term assistance following discharge from a hospital or rehabilitation facility while the individual stabilizes.

KEY POINTS TO COMMUNICATE TO FAMILIES

Staff must clearly explain that:

- AAA services are time-limited and will end.
- Once services conclude, care must transition to:
 - Private Pay (PP)
 - Long-Term Care Insurance (LTC)
 - Another payer source or provider that accepts the client's insurance

3 ELIGIBILITY REQUIREMENTS

To qualify for an AAA referral, the individual must:

- Reside in one of the approved counties listed on the left
- Be 60 years of age or older
- Not have Medicaid
- Meet income eligibility requirements (refer to the income limits listed on the AAA referral page)



4 REFERRAL SUBMISSION PROCESS

STEP 1 DETERMINE REFERRAL TYPE

Complete the AAA referral form and identify the appropriate program.

A. CAREGIVER SUPPORT

Applies to individuals 55 years or older who meet any of the following criteria:

- Provide care for a loved one over the age of 60
- Miss work hours to care for someone over 60
- Care for both minors and a loved one over 60
- Have personal medical conditions while caring for someone over 60

B. CARE COORDINATION

Applies to individuals who:

- Meet eligibility requirements (age, county, no Medicaid, limited access to private pay funds), and
- Are 60 years of age or older

STEP 2 COMPLETE REQUIRED REFERRAL FORM SECTIONS

The following information must be included at minimum:

- PAGE 1**
 - Date
 - Referral Source
 - Name of individual needing services
 - Ensure all applicable service boxes are checked
- PAGE 2**
 - All fields marked with an asterisk (*) must be completed.*
 - Confirm the top-right checkbox is selected
 - Explain and confirm:
 - Release of Information
 - Client Rights and Responsibilities
 - Required details include:
 - Full Name
 - Date of Birth
 - Address
 - Gender
 - Phone Number
 - Ethnicity
 - Race
 - Living situation (Does the individual live alone?)
 - Income status (at or below poverty level)
- PAGE 3**
 - Diagnoses and/or medical conditions
 - Reason(s) services are being requested
 - Emergency or primary contact name
 - Primary Care Provider (PCP)
 - Referral Source

STEP 3 SAVE AND DISTRIBUTE REFERRAL

Download and save the completed referral using the following naming convention:

"AAA Referral - [Client Name] - [Date]"

Upload the saved referral to:

- The prospect's profile
- The CAPCOG referral profile

Email the completed referral to ccinfo@capcog.org and CC Elizabeth and Anna Rose

5 FAMILY COMMUNICATION GUIDELINES

Inform families that AAA referrals typically take 4-6 weeks to process.

Advise families that private pay services may begin at any time if care is needed before AAA approval.

6 FOLLOW-UP PROCEDURE

FOLLOW-UP #1 2 WEEKS POST-SUBMISSION

Create a follow-up task for yourself and Elizabeth to assess:

- Current status
- Whether the family wishes to wait for AAA or move forward with Private Pay services

FOLLOW-UP #2 3-4 WEEKS POST-SUBMISSION

Create an additional follow-up task to:

- Check for AAA contact
- Obtain any status updates from the family

COMPLIANCE NOTE

Accurate completion, documentation, and timely follow-up are required to ensure compliance with AAA referral standards and internal Right at Home procedures.

REMEMBER: COMPLETE • DOCUMENT • COMMUNICATE • FOLLOW UP

Ensure the individual meets all eligibility requirements.

Complete every required section of the referral form accurately.

Distribute to all required recipients and save in the correct locations.

Follow up on time and keep the family informed throughout the process.

GUIDE Program

Dementia care authorization, eligibility, and private pay profile requirements

Who Qualifies for GUIDE

- Confirmed dementia or Alzheimer's diagnosis
- Enrolled in traditional Medicare (NOT Medicare Advantage)
- Not currently on Medicare hospice services
- Not residing in a long-term nursing home as primary residence
- Not already enrolled in GUIDE through another provider group

Who Does NOT Qualify

- Enrolled in Medicare Advantage or another managed Medicare plan
- Actively receiving Medicare hospice services
- Long-term nursing home is their primary residence
- Already enrolled in GUIDE through a different provider

Authorization Timeline & Hours

- Authorization takes approximately 6 weeks to process
- 72 hours authorized across the board, must be used in 4-hour shifts minimum, deadline 6/30/26
- New authorization will be issued effective 7/1/26 if eligible

Delays occur when family misses GUIDE calls, cancels/reschedules repeatedly, or is slow to provide documentation

GUIDE. Private Pay Profile Requirement

Section 03

GUIDE clients using Private Pay MUST have a SEPARATE Wellsky profile - billing cannot process without it

3 Requirements Before Private Pay Starts

- 1. Separate Wellsky profile, name format: 'First Last GUIDE'
- 2. ACH information from the client
- 3. New signed service agreement (separate from the GUIDE auth agreement)
- Without ALL THREE, private pay billing cannot be processed

Scheduling Rules for GUIDE

- Reschedule ANY time within the authorization period — no weekly or monthly restrictions
- All shifts must be a minimum of 4 hours — no shorter shifts are allowed under GUIDE authorization
- 72 hours total across the auth period — once auth hours are exhausted, transition to private pay (notify client first)

GUIDE Referral - Staff Intake Workflow

Section 03

1

STEP 1 • Treat as standard inquiry call

Gather as much information as possible. Focus on: photo of Medicare card, ADL Assessment (basic is acceptable, enter under Client Profile - Forms), Home Safety Assessment

2

STEP 2 • Gather neurologist info (preferred)

Name of neurologist, office name and phone number. OR any supporting documentation confirming a dementia/Alzheimer's diagnosis. Not mandatory but significantly reduces processing delays

3

STEP 3 • Send all documents to Anna Rose or Elizabeth

Forward everything collected, they are responsible for uploading documents to the drive and scheduling the GUIDE call with the family

4

STEP 4 • Place a same-day follow-up task

Assign a task to Anna Rose AND Elizabeth the same day confirming that documents are ready for upload and call scheduling. Do not assume they received them without a task

5

STEP 5 • Encourage family to attend ALL GUIDE calls

Missed calls, repeated cancellations, and slow documentation are the #1 causes of delay beyond the standard 6-week timeline

Intake staff: information gathering ONLY. Anna Rose & Elizabeth handle all uploading and GUIDE call scheduling until further notice.

GUIDE PROGRAM – TRAINING GUIDE

In-Home Support • Caregiver Relief • Temporary Assistance



1 OVERVIEW

The GUIDE (Guided Unobtrusive In-Home Dementia Engagement) Program provides in-home support services for individuals diagnosed with dementia and a respite (temporary relief) for their primary caregivers.



2 SERVICE GOALS

- ✓ Improve quality of life and safety at home.
- ✓ Reduce caregiver stress by providing temporary relief.
- ✓ Support independence and engagement for individuals with dementia.

3 ELIGIBILITY REQUIREMENTS

- Individual must be diagnosed with dementia.
- Primary caregiver must live in the same household.
- Must meet financial eligibility requirements.
- Must reside within the approved service area.

4 AUTHORIZED SERVICES



IN-HOME SUPPORT SERVICES

- Companionship and engagement
- Assistance with activities of daily living (ADLs): bathing, dressing, toileting, mobility, eating
- Light housekeeping related to the individual's needs
- Meal preparation
- Medication reminders (no administration)
- Safety monitoring and redirection
- Transportation to medical appointments (pre-approved)

RESPITE (TEMPORARY RELIEF) SERVICES

- Short-term relief for the primary caregiver
- Services provided in the home
- Can be scheduled on a recurring or as-needed basis (subject to program approval)
- Supports caregiver rest, appointments, or emergency needs

5 REFERRAL PROCESS



1. REFERRAL
Referral received from approved source (healthcare provider, AA, partner agency, or self-referral).



2. SCREENING
Program staff conducts initial screening to confirm eligibility and collect required information.



3. ASSESSMENT
Comprehensive assessment of individual and caregiver needs is completed.



4. APPROVAL
Eligibility and service plan are reviewed and approved.



5. START OF SERVICES
Services begin based on the approved plan and schedule.

6 SERVICE PLANNING



- A personalized service plan will be created based on needs assessment.
- Services and schedules may change as needs evolve.
- Caregiver input is essential in developing the plan.

7 CAREGIVER RESPONSIBILITIES



- Be available for assessments and care planning.
- Communicate changes in needs or circumstances.
- Ensure the safety of the individual and support staff.
- Provide feedback on services.
- Notify program staff of hospitalizations or significant changes.

8 PARTICIPANT RESPONSIBILITIES



- Participate in services as outlined in the plan.
- Treat staff with respect.
- Allow staff to provide agreed-upon support.
- Follow safety guidelines.

9 STAFF RESPONSIBILITIES



- Provide services according to the care plan.
- Document all visits accurately.
- Communicate with caregiver and program staff.
- Maintain confidentiality and professional standards.

10 DOCUMENTATION & REPORTING



Accurate documentation of all services provided is required.



Record start/end times, services provided, and any changes.



Report incidents or concerns immediately to the program supervisor.



Maintain confidentiality of all participant information.

11 TERMINATION OF SERVICES



- Services may be ended if:
- The individual no longer meets eligibility.
 - The caregiver no longer needs services.
 - There is non-compliance with program policies.
 - The participant moves out of the service area.

12 IMPORTANT POLICIES

- Services are non-medical and not a substitute for medical care.
- Abuse, neglect, or exploitation will not be tolerated.
- Program staff are not allowed to administer medications.
- Program funding is limited and services are not guaranteed beyond available resources.

13 CONTACT & SUPPORT



For questions or concerns, contact your program coordinator or supervisor.



In case of emergency, contact local emergency services or your program supervisor immediately.



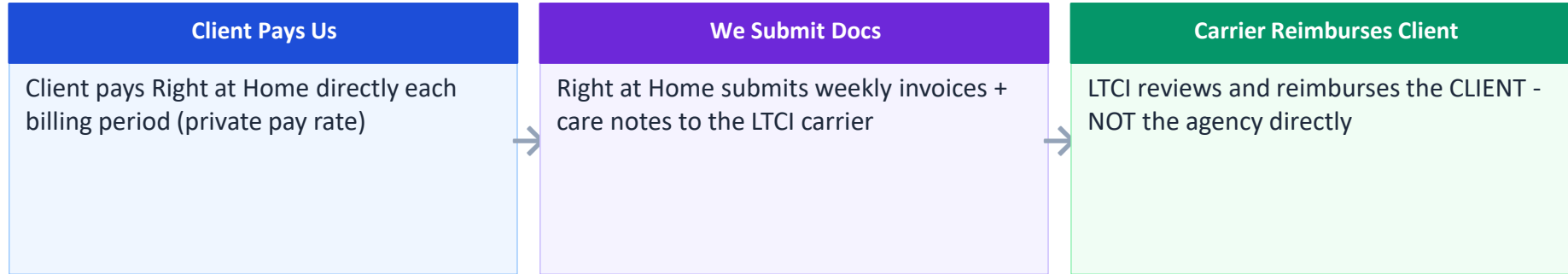
OUR MISSION: To support individuals with dementia and their caregivers by providing compassionate in-home support and temporary relief, promoting dignity, safety, and independence.

Long-Term Care Insurance (LTC)

How LTCI works, what to ask on intake calls, and how to educate clients

LTC Insurance - How It Works & Intake Questions

Section 04



Questions to Ask on Every LTCI Inquiry Call
• Who is the LTCI policy with? (carrier name) • Policy number, who is the policyholder (client or spouse)? • What is the elimination period? (30, 60, or 90 days most common, client pays privately during this period before benefits begin) • How many ADLs must be met to activate the policy? (usually, 2 of 6 ADLs OR cognitive impairment) • What is the benefit amount, daily or monthly maximum?

Critical Reminders for Staff
• Do NOT say or imply insurance pays Right at Home directly it doesn't • Do NOT guarantee reimbursement timelines every carrier is different • Document that reimbursement process was explained to the client • If client expresses financial concern - flag for leadership before starting care; an AAA referral may help cover the elimination period • Refer to Stana Martin ('Mrs. LTC') - expert at LTCI navigation and claims filing



LONG-TERM CARE INSURANCE (LTCI)

EXPLAINING HOW IT WORKS

A training guide to understand Long-Term Care Insurance and how it works with Right at Home.



1 WHAT IS LONG-TERM CARE INSURANCE?

Long-Term Care Insurance (LTCI) is designed to help cover the cost of long-term care services when someone needs assistance with daily activities due to aging, illness, injury, or cognitive decline.



LTCI focuses on custodial & supportive care, not medical treatment.



Helps pay for care at home, in a community, or in a facility.



Provides financial protection and peace of mind.



Helps preserve savings and assets.



Policies vary—know your benefits.

2 KEY INFORMATION TO GATHER

Questions to Ask on the Inquiry Call



1 LTC Insurance Provider

"Who is your long-term care insurance policy with?"



2 Policy Number

"Do you happen to have your policy number available?"
• Note who the policyholder is (client or spouse).



3 Elimination Period

"Do you know what your elimination period is?"
• The elimination period is the number of days care must be paid for privately before benefits begin.
• Common: 30, 60, or 90 days



4 ADL Requirement

"Do you know how many Activities of Daily Living need to be met for your policy to activate?"
• Most policies require 2 of 6 ADLs or a cognitive impairment



5 Benefit Amount

"Do you know what your coverage amount is—daily or monthly?"

3 HOW LONG-TERM CARE INSURANCE WORKS: THE GENERAL PROCESS

1. CARE IS NEEDED



The client needs assistance with daily activities due to aging, illness, or cognitive decline.

2. ELIMINATION PERIOD



The client pays privately for care during the elimination period (e.g., 30, 60, or 90 days).

3. POLICY ACTIVATION



Once the elimination period is met and ADL requirements are satisfied, the policy becomes active.

4. CARE RECEIVED



The client receives care services from Right at Home.

5. PAYMENT TO RIGHT AT HOME



The client pays Right at Home directly for services received.

6. REIMBURSEMENT TO CLIENT



The insurance company reimburses the client after reviewing and approving the claim.

☆ **IMPORTANT:** Insurance reimburses the client, not Right at Home.

4 WHAT CLIENTS NEED TO KNOW



LTCI is reimbursement-based coverage.

Clients pay Right at Home directly. The insurance company reimburses the client.

What We (Right at Home) Will Do:

- ✓ Submit weekly invoices
- ✓ Submit weekly care notes
- ✓ Assist with required documentation for the insurance carrier
- ✓ Reimbursement occurs after the insurance company reviews and approves the submitted documentation

Example

Elimination Period: 60 days
Daily Rate: \$200

- Client pays privately for 60 days (60 x \$200 = \$12,000)
- After 60 days and approval, LTCI reimburses the client based on the policy terms.

5 IMPORTANT STAFF REMINDERS



DO NOT state or imply that the insurance pays Right at Home directly.



DO NOT guarantee reimbursement timelines (each carrier differs).



Document that reimbursement expectations were explained.



If the client expresses financial concern, flag the account for leadership review before starting care.



Offer to refer them to Stana Martin, "Mrs. LTC". She is an expert at navigating LTC policies and can even help file claims.

6 COMMON TERMS TO KNOW



Elimination Period: The number of days care must be paid for privately before the policy benefits begin.



ADLs (Activities of Daily Living): Basic tasks such as bathing, dressing, eating, toileting, transferring, continence, and mobility. Most policies require 2 of 6 ADLs or a cognitive impairment.



Benefit Amount: The maximum the policy will pay (daily or monthly) for approved services.



Policy Activation: When the elimination period is met and policy requirements are satisfied, benefits begin.



TALKING POINT FOR STAFF

"Long-term care insurance works on a reimbursement basis. You would pay us directly, and then your insurance company reimburses you after we submit invoices and care notes each week. We help support you through that entire process."



KEY TAKEAWAY

Our role is to educate, collect accurate information, set clear expectations, and support the client through the reimbursement process to ensure a smooth experience.



OUR MISSION: To improve the quality of life for those we serve.

Compassion • Trust • Excellence

VA Personal Care Services (PCS)

H/HHA, Respite Care, authorized ADLs, prohibited tasks, and compliance

VA PCS - Program Overview: H/HHA vs. Respite

Section 05

H/HHA - Homemaker / Home Health Aide

- Personal care assistance for daily safety at home
- Core services: bathing, dressing, grooming, toileting, transfers, eating assistance
- Must be ordered by a VA physician based on medical necessity
- Requires a CONSISTENT AUTHORIZED SCHEDULE (e.g., 2 hrs/day, 5 days/week)
- Hours cannot be reallocated or combined between days, fixed schedule only

Respite Care

- Provides temporary relief for the Veteran's primary caregiver
- Approved as a WEEKLY BLOCK of hours (e.g., 10 hours/week)
- Flexible scheduling between Veteran and agency within the week
- CAN stack Respite shifts on top of other Respite shifts
- NO CARRYOVER, unused weekly hours do not roll over to the next week
- Reschedule within same WEEK only

Authorized Activities of Daily Living (ADLs) - Each task must be documented individually

Personal Care

- Bathing
- Dressing (shirts/pants/undergarments)
- Grooming: shaving, teeth brushing, hair combing

Mobility

- Bed mobility: turning/repositioning
- Transfers: in/out of bed, sitting/standing
- Walking and ambulation assistance

Nutrition / Toileting

- Feeding assistance (NOT meal prep)
- Toileting transfers
- Toileting hygiene support

✕ Prohibited Tasks — Never Authorized Under VA PCS

- Medications: Administration or setup is strictly prohibited — verbal reminders only are permitted
- Housekeeping: General cleaning is not authorized; light housekeeping in care areas at the END of a shift only
- Transportation: Errands or accompanying Veterans to appointments is strictly prohibited (no exceptions)
- Other: Pet care, yard work, or any outdoor maintenance tasks

📄 Compliance Requirements

- EVV (Electronic Visit Verification) is MANDATORY for every visit — no exceptions
- Discrepancy between EVV and billing will result in non-payment by VA
- Each ADL task must be documented individually in visit notes
- Records must reflect actual assistance provided — not templated
- Agencies must perform periodic supervision visits and ensure staff competency
- Annual training and TB testing required for all caregivers

🔔 Reporting & Start of Care

- Health or safety incidents must be reported to VA within 24–48 hours
- Changes in the Veteran's condition must be reported promptly via secure portal
- Referral → Agency confirms acceptance by next business day
- VA coordinates start date — Initial Plan of Care (IPOC) must be submitted within 7 business days of first visit
- All billing and clinical communications must use secure portals to protect PHI

VA VA PERSONAL CARE SERVICES (PCS) OPERATIONS MANUAL – TRAINING GUIDE

Supporting Veterans. Supporting Caregivers. Supporting Independence at Home.



1 PROGRAM OVERVIEW

The VA Personal Care Services (PCS) programs are designed to support Veterans in living safely at home while providing relief to their primary caregivers.

HOMEMAKER / HOME HEALTH AIDE (H/HHA)



- Provides personal care assistance for daily safety at home.
- Core Services:** Bathing, dressing, grooming, toileting, transfers, and assistance with eating.



REQUIREMENT: Must be ordered by a VA physician based on medical necessity.

RESPITE CARE SERVICES



- Provides temporary relief for the Veteran's caregiver.
- Can be provided in-home or in inpatient settings (e.g., VA CLC).
- Typically "sitter-type" services; ADL assistance is provided only as needed during the period.

2 AUTHORIZED ACTIVITIES OF DAILY LIVING (ADLs)

PCS staff may provide assistance with the following authorized ADL tasks:



PERSONAL CARE

Bathing, dressing (including shirts/pants/undergarments), and grooming (shaving, teeth brushing, hair combing).



MOBILITY

Bed mobility (turning/repositioning), transfers (in/out of bed, sitting/standing), and walking/ambulation.



NUTRITION / TOILETING

Feeding assistance (excluding meal prep) and toileting transfers/hygiene support.

3 REFERRAL AND START OF CARE PROCESS

1 REFERRAL

Veterans apply through their VA Primary Care Team. VA PCS staff conduct clinical screening for approval.

2 AGENCY NOTIFICATION

Agencies are contacted via phone/email and must confirm acceptance by the next business day.

3 START DATE

VA coordinates the start date. The Initial Plan of Care (IPOC) must be submitted to the VA within 7 business days.

4 SCHEDULING AND HOUR DETERMINATION

H/HHA RULES



- Hours are based on the Case Mix Tool Assessment.
- Requires a Consistent Authorized Schedule (e.g., 2 hours/day, 5 days/week).
- Hours cannot be reallocated or combined (e.g., cannot move hours from one day to another).

RESPITE RULES



- Approved as a Weekly Block of Hours (e.g., 10 hours/week).
- Allows Flexible Scheduling between the Veteran and the agency.
- No Carryover:** Weekly hours cannot roll over to the next week.

5 DOCUMENTATION, EVV, AND COMPLIANCE



EVV (ELECTRONIC VISIT VERIFICATION)

Mandatory for all visits. Discrepancies between EVV and billing will result in non-payment.



DOCUMENTATION
Each ADL task must be documented individually. Records must reflect actual assistance provided.



SUPERVISION
Agencies must perform periodic supervision and ensure staff competency (annual training and TB testing).



INCIDENT REPORTING
Changes in health or safety incidents must be reported to the VA within 24–48 hours.



COMPLIANCE
Follow all VA policies, program requirements, and standards to ensure Veteran safety and program integrity.

6 PROHIBITED TASKS (NON-AUTHORIZED)



MEDICATION
Administration or setup is prohibited. Verbal reminders only.



HOUSEKEEPING
General cleaning is not authorized; light housekeeping only in care areas at the end of a shift.



TRANSPORTATION
Errands or accompanying Veterans to appointments is strictly prohibited.



OTHER
Pet care, yard work, and outdoor maintenance are not authorized.

7 CONTACT AND SUPPORT



Communication regarding authorizations, billing, or clinical changes must be conducted via secure portals to protect PHI and ensure compliance.



PCS PROGRAM SUPPORT
Contact your local VA PCS Representative for questions or assistance.



SECURE COMMUNICATION
Use VA-approved systems and secure portals only. Do not email or share PHI through unsecured methods.



WE'RE HERE TO HELP
Our goal is to support Veterans, caregivers, and agencies in providing high-quality, compliant care every day.

Program Summary

- AAA: temporary program, set transition expectations from day one; never imply extensions are possible
- GUIDE: traditional Medicare required; 72-hour auth; 4-hr shifts minimum; separate Wellsky profile + ACH + service agreement for private pay
- LTC Insurance: reimbursement-based, client pays us, insurance reimburses the client; document reimbursement explanation
- VA H/HHA: fixed schedule, same-month rescheduling, no same-day stacking with another H/HHA shift
- VA Respite: flexible weekly block, stacking allowed, no carryover, same-week rescheduling only
- All VA programs: EVV mandatory, no errands, no medications, document every ADL individually